

TOP 10 HERBS FOR HYPERTENSION

*The Deep Dive: Mechanisms, Dosing, Clinical Studies,
Contraindications, and Practical Usage for Each One*

God's pharmacy was open long before CVS.

10

evidence-backed herbs for
blood pressure

50+

clinical studies referenced

6

drug classes mimicked naturally

0

prescriptions required

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DISCLAIMER: This guide is for educational purposes only. Do not discontinue any prescribed medication without speaking to your doctor.
Always consult your healthcare provider before starting any supplement.

#1 | HIBISCUS (Hibiscus sabdariffa)

Natural ACE Inhibitor + Diuretic

"The Red Flower That Rivals Your Prescription"

QUICK FACTS	
Drug class it mimics:	ACE Inhibitors (Lisinopril, Enalapril) + Diuretics (HCTZ)
Active compounds:	Anthocyanins, polyphenols, organic acids (hibiscus acid, hydroxycitric acid)
Recommended dose:	2-3 cups daily (steep 5-10 min with lid on) OR 1-2g dried calyx powder
BP reduction:	Systolic: -7.1 to -7.6 mmHg avg Diastolic: -3.3 to -3.8 mmHg avg (meta-analyses)
Time to effect:	Measurable within 2-4 weeks of daily use
Comparable to:	Captopril 25mg in one clinical trial No significant difference from medication in meta-analysis

HOW IT WORKS:

Hibiscus contains anthocyanins — the pigments responsible for its deep red color — that directly inhibit angiotensin-converting enzyme (ACE). This is the **exact same target** as lisinopril, enalapril, and ramipril. By blocking ACE, angiotensin II production decreases, blood vessels relax, and pressure drops. Simultaneously, hibiscus triggers nitric oxide release from the vascular endothelium, which enhances vasodilation further. It also acts as a mild natural diuretic — promoting kidney filtration and sodium excretion **WITHOUT** flushing potassium like pharmaceutical diuretics do.

THE KEY STUDIES:

A 2022 meta-analysis of 26 RCTs (1,797 participants) found hibiscus lowered SBP by -7.1 mmHg vs placebo. A dose-response analysis showed that for each additional gram of hibiscus consumed, SBP dropped an additional -6.45 mmHg. A Tufts University RCT (65 adults, 6 weeks) found 3 cups/day of hibiscus tea lowered SBP by -7.2 mmHg vs -1.3 for placebo. A 2025 umbrella review of 26 RCTs confirmed BP-lowering effects comparable to antihypertensive drugs, with no significant difference between hibiscus and medication groups.

HOW TO USE IT:

Steep dried hibiscus flowers or quality tea bags in hot water for 5-10 minutes **WITH A LID** on the cup (this traps the volatile oils that would otherwise evaporate). Drink 2-3 cups throughout the day — morning, afternoon, and evening. Can be consumed hot or iced. For maximum potency, use loose dried hibiscus calyces rather than pre-made tea bags. The deeper the red color, the more anthocyanins are present.

CAUTIONS: May interact with ACE inhibitors (additive effect — monitor BP). May lower BP too much if combined with multiple BP medications. Avoid during pregnancy in medicinal doses. Can interact with chloroquine and acetaminophen absorption.

#2 | GARLIC (*Allium sativum*)

*ACE Inhibitor + CCB + Alpha-Blocker
+ Vasodilator*

"The Multi-Mechanism King — 4 Drug Classes in One Clove"

QUICK FACTS	
Drug classes it mimics:	ACE Inhibitor + Calcium Channel Blocker + Alpha-Blocker + Vasodilator
Active compounds:	Allicin, S-allylcysteine, ajoene, diallyl sulfides
Recommended dose:	2 raw cloves/day (crushed, wait 10 min) OR 600-1200mg aged garlic extract
BP reduction:	Systolic: -8.3 mmHg avg Diastolic: -5.5 mmHg avg (meta-analysis)
Time to effect:	2-4 weeks for full ACE inhibition. Nitric oxide boost begins within hours.
Comparable to:	Atenolol 50mg in one study. 16-40% cardiovascular risk reduction (2020 review).

HOW IT WORKS:

Garlic is the only plant documented to work through FOUR separate antihypertensive mechanisms simultaneously. (1) ACE Inhibition: allicin and S-allylcysteine block angiotensin-converting enzyme. (2) Nitric Oxide + Hydrogen Sulfide: garlic stimulates both NO and H₂S production — two of the body's most powerful vasodilators. (3) Calcium Channel Blocking: garlic compounds inhibit calcium influx into vascular smooth muscle, preventing contraction. (4) Alpha-Adrenergic Blocking: at higher doses, garlic blocks alpha receptors that cause vasoconstriction. No single pharmaceutical drug targets all four of these pathways.

THE KEY STUDIES:

A meta-analysis found garlic supplementation reduced SBP by -8.3 mmHg and DBP by -5.5 mmHg on average. A 2020 review found garlic supplementation lowered BP enough to reduce cardiovascular risk by 16-40%. S-allylcysteine combined with captopril showed synergistic BP reduction in clinical research — meaning garlic enhances the effect of ACE inhibitor medication rather than competing with it. Iranian traditional medicine research confirmed garlic's ACE inhibition, NO stimulation, H₂S production, and calcium channel blocking in both animal and human studies.

HOW TO USE IT:

Raw garlic (most potent): CRUSH 2 cloves with the flat of a knife and LET SIT for 10 minutes. This is non-negotiable — the enzyme alliinase needs time to convert alliin into allicin. Without those 10 minutes, you are eating a vegetable, not a vasodilator. Eat with food. Chase with honey if needed. **Aged garlic extract (Kyolic):** 600-1200mg daily in divided doses. Most studied supplement form. Gentler on stomach. S-allylcysteine is the primary active compound in aged extract.

CAUTIONS: Blood-thinning effect — use caution with warfarin, aspirin, or other anticoagulants. May increase bleeding risk before surgery (stop 7-10 days before). Can cause GI upset in some people — take with food. May enhance effect of BP medications — monitor readings.

#3 | MAGNESIUM

Natural CCB + Vasodilator + Muscle Relaxant

"Nature's Original Calcium Channel Blocker"

QUICK FACTS	
Drug class it mimics:	Calcium Channel Blockers (Amlodipine, Felodipine, Nifedipine)
Active form:	Elemental magnesium — delivered via glycinate, taurate, citrate, or topical (Epsom salt)
Recommended dose:	300-400mg elemental magnesium daily (glycinate or taurate preferred)
BP reduction:	Systolic: -5.6 mmHg avg Diastolic: -2.8 mmHg avg (meta-analysis of 34 trials)
Time to effect:	Days for muscle relaxation. 2-4 weeks for full BP impact.
Deficiency rate:	Over 50% of Americans are magnesium deficient. Up to 80% of hypertensives.

HOW IT WORKS:

Magnesium competes with calcium at the voltage-gated calcium channels in vascular smooth muscle cells. This is the **identical mechanism** as pharmaceutical CCBs like amlodipine. When calcium enters these cells, the muscle contracts and the vessel narrows. Magnesium blocks this calcium influx, relaxing the muscle and dilating the vessel. Beyond this, magnesium is a cofactor in over 300 enzymatic reactions — including energy production, nerve function, blood glucose control, and the production of nitric oxide. It also activates the parasympathetic nervous system and regulates melatonin production, meaning it supports sleep quality — which directly improves nocturnal BP dipping.

THE KEY STUDIES:

A meta-analysis of 34 clinical trials (2,028 participants) published in Hypertension found magnesium supplementation reduced SBP by -5.6 mmHg and DBP by -2.8 mmHg. The effect was strongest in people who were magnesium-deficient at baseline — which is over half the population. A study in the Journal of Research in Medical Sciences found magnesium significantly improved sleep quality, sleep time, and sleep onset — all of which improve nocturnal BP dipping. Soil depletion has reduced magnesium content in food by up to 80% over the last century.

HOW TO USE IT:

Magnesium Glycinate: Best absorbed, least GI upset, calming. Ideal for evening/bedtime. **Magnesium Taurate:** Specifically researched for cardiovascular health. Taurine itself supports heart function. **Magnesium Citrate:** Well-absorbed but mild laxative effect. **AVOID Magnesium Oxide:** Only 4% bioavailability. Cheapest on the shelf but least effective. **Topical:** Epsom salt bath — 2 cups in warm water, soak 20 minutes. Magnesium absorbs through skin. Take oral forms with dinner or before bed for best results.

CAUTIONS: Kidney disease patients should consult their doctor (kidneys regulate Mg excretion). High doses can cause loose stools — reduce dose or switch to glycinate. May enhance effect of CCB medications.

#4 | HAWTHORN BERRY (Crataegus)

*Natural Beta-Blocker + Cardiac Tonic**"The Heart Herb — 2,000 Years of Cardiovascular Use"*

QUICK FACTS	
Drug class it mimics:	Beta-Blockers (Metoprolol, Atenolol, Propranolol)
Active compounds:	Oligomeric proanthocyanidins (OPCs), vitexin, hyperoside, quercetin
Recommended dose:	250-500mg standardized extract 2-3x daily OR 1-2 tsp dried berries as tea
BP reduction:	Variable; 2020 systematic review confirmed BP-lowering and heart rate reduction
Time to effect:	4-8 weeks for full effect. Slow onset but sustained benefit.
History:	Used for cardiovascular support since 1st century AD. One of the most studied herbs.

HOW IT WORKS:

Hawthorn acts on the cardiovascular system similarly to beta-blockers — it slows heart rate, reduces the force of cardiac contractions, and decreases myocardial oxygen demand. But unlike pharmaceutical beta-blockers, it also **STRENGTHENS** the heart muscle over time through improved coronary blood flow and enhanced cellular energy production. Its OPCs (oligomeric proanthocyanidins) are potent antioxidants that protect the vascular endothelium from oxidative damage. Hawthorn also inhibits ACE to a mild degree and enhances nitric oxide availability.

THE KEY STUDIES:

A 2020 systematic review confirmed hawthorn's ability to lower both blood pressure and heart rate. A Cochrane review of hawthorn for heart failure found significant improvement in exercise tolerance and reduction in symptoms (shortness of breath, fatigue). Multiple European studies have demonstrated hawthorn's cardioprotective effects, and it is approved as a traditional herbal medicine for cardiac insufficiency in Germany. Unlike beta-blockers, hawthorn does not cause fatigue, weight gain, or sexual dysfunction.

HOW TO USE IT:

Tea: Steep 1-2 teaspoons of dried hawthorn berries in boiling water for 15-20 minutes (longer steep = stronger extraction). Drink 2-3 cups daily. **Capsules:** 250-500mg of standardized extract (1.8% vitexin or 18.75% OPCs), taken 2-3 times daily with meals. **Tincture:** 1-2ml, 3 times daily. Hawthorn is a slow-building herb — give it 4-8 weeks before judging effectiveness.

CAUTIONS: May enhance effects of cardiac glycosides (digoxin). May potentiate beta-blockers and antihypertensive drugs. Start low if on heart medication. Generally very well-tolerated with an excellent safety profile.

#5 | BEETROOT / BEET JUICE

Natural CCB via Nitric Oxide Pathway

"The Nitric Oxide Powerhouse — Works Within Hours"

QUICK FACTS	
Drug class it mimics:	Calcium Channel Blockers (Amlodipine) via nitric oxide vasodilation
Active compounds:	Dietary nitrate (converted to nitric oxide), betalains, betaine
Recommended dose:	250ml (~8.5oz) beet juice daily OR 500mg beetroot powder supplement
BP reduction:	Systolic: -4 to -8 mmHg avg Some studies show effect within 3-6 HOURS
Time to effect:	Acute: measurable within hours. Sustained: builds over days-weeks.
Key study:	Queen Mary University of London — BP dropped within hours of single dose

HOW IT WORKS:

Beetroot is one of the richest dietary sources of inorganic nitrate. When you consume it, bacteria on your tongue convert nitrate to nitrite, which is then converted to nitric oxide (NO) in your blood. NO is the master vasodilator — it signals vascular smooth muscle to relax, arteries dilate, and blood pressure drops. This is the same vasodilation pathway targeted by calcium channel blockers, but through a different entry point. Beetroot also contains betalains (the red pigments) which are powerful anti-inflammatory and antioxidant compounds that protect the endothelial lining.

THE KEY STUDIES:

The landmark Queen Mary University of London study found that a single glass of beet juice lowered blood pressure within hours, with effects lasting up to 24 hours. Average reduction: -8/4 mmHg in hypertensive participants. A meta-analysis of 22 trials confirmed beet juice significantly reduces both systolic and diastolic BP, with the strongest effects in people with hypertension. The nitrate-to-nitric oxide pathway is now one of the most researched areas in cardiovascular nutrition.

HOW TO USE IT:

Juice: 250ml (one glass) of raw beet juice daily. Can be mixed with apple, carrot, or ginger to improve taste. **Powder:** 500mg-1g beetroot powder mixed into water or smoothie. **Whole beets:** Roasted, steamed, or raw in salads. Cooking reduces nitrate content somewhat — raw or juiced is most potent. **Pro tip:** Do NOT use antibacterial mouthwash before drinking beet juice — it kills the tongue bacteria that convert nitrate to nitrite. No bacteria = no conversion = no benefit.

CAUTIONS: May cause red urine/stool (harmless — called beeturia). High in oxalates — caution with kidney stone history. May enhance effects of BP medications. High in natural sugars — diabetics monitor glucose.

#6 | CoQ10 (Coenzyme Q10 / Ubiquinone)

Nitric Oxide Preserver +
Mitochondrial Support

"The Heart's Energy Source That Your Meds Are Depleting"

QUICK FACTS	
Drug class it supports:	Counters depletion caused by Beta-Blockers, Statins, and Diuretics
Active form:	Ubiquinone (standard) or Ubiquinol (reduced, more bioavailable)
Recommended dose:	100-200mg daily (higher if on statins or beta-blockers)
BP reduction:	All 12 clinical trials in meta-analysis showed BP reduction
Time to effect:	4-12 weeks for measurable BP impact
Critical fact:	Beta-blockers AND statins actively deplete CoQ10 from your body

HOW IT WORKS:

CoQ10 is not an herb — it is a naturally occurring compound essential for mitochondrial energy production. Your heart, as the hardest-working muscle in your body, has the highest CoQ10 demand of any organ. CoQ10 lowers blood pressure by preserving nitric oxide from destruction by free radicals. When free radicals inactivate NO, vessels cannot relax properly. CoQ10's antioxidant action protects NO, keeping vessels dilated. It may also boost prostacyclin production — a potent vasodilator and platelet inhibitor. Critically, beta-blockers, statins, and some diuretics actively DEplete CoQ10 from your body — creating a vicious cycle where the medication that treats your symptom worsens the underlying deficiency.

THE KEY STUDIES:

A meta-analysis of 12 clinical trials found CoQ10 supplementation reduced blood pressure in every single trial — whether used alone or alongside standard medication. The Q-SYMBIO trial (420 heart failure patients, 2 years) found CoQ10 supplementation reduced cardiovascular mortality by 43% and all-cause mortality by 42%. Multiple studies confirm that statins reduce plasma CoQ10 levels significantly, and that supplementation can restore levels and reduce statin-related muscle pain and fatigue.

HOW TO USE IT:

Take 100-200mg daily with a meal containing fat (CoQ10 is fat-soluble). If on statins or beta-blockers, the higher end (200mg) is recommended. **Ubiquinol** is the reduced, more bioavailable form — preferred for anyone over 40 or with absorption concerns. **Ubiquinone** is the standard form and is effective for most people. Take in divided doses (100mg morning + 100mg evening) for best absorption.

CAUTIONS: May reduce effectiveness of warfarin (blood thinner) — monitor INR. May lower blood sugar — diabetics monitor glucose. Generally very safe with minimal side effects.

#7 | OLIVE LEAF EXTRACT (*Olea europaea*)

Natural ACE Inhibitor + CCB +
Beta-Blocker Enhancer

"The Biblical Herb That Rivals Captopril"

QUICK FACTS	
Drug classes it mimics:	ACE Inhibitor + Calcium Channel Blocker + enhances Beta-Blockers
Active compounds:	Oleuropein, hydroxytyrosol, oleacein
Recommended dose:	500mg standardized extract (min 20% oleuropein) twice daily
BP reduction:	One trial found comparable to Captopril 25mg at 500mg twice daily
Time to effect:	2-4 weeks for measurable BP reduction
Biblical significance:	The olive branch is one of the most referenced plants in Scripture

HOW IT WORKS:

Olive leaf extract contains oleuropein — a compound that breaks down into molecules with direct ACE-inhibiting activity. Animal studies have also demonstrated calcium channel blocking properties and vasodilatory effects. Uniquely, olive leaf extract has been shown to enhance the effects of three major drug classes simultaneously — ACE inhibitors, calcium channel blockers, AND beta-blockers. This makes it an ideal natural companion to pharmaceutical treatment during the transition to lower medication doses.

THE KEY STUDIES:

A clinical trial comparing olive leaf extract (500mg twice daily) to Captopril (25mg twice daily) found comparable blood pressure reduction over 8 weeks. Olive leaf also improved triglyceride levels — an effect Captopril did not produce. Life Extension research confirmed olive leaf's dual ACE-inhibiting and calcium channel blocking properties, noting it provides two of the most effective drug mechanisms in a single supplement.

HOW TO USE IT:

Take 500mg of standardized olive leaf extract (minimum 20% oleuropein) twice daily with meals. Liquid extract and tea forms are also available but harder to standardize for consistent dosing. Can be combined with celery seed extract for a natural ACE inhibitor + CCB combination.

CAUTIONS: May enhance effects of BP medications (monitor readings). May lower blood sugar — diabetics monitor glucose. Generally well-tolerated. Avoid during pregnancy in supplement doses.

#8 | DANDELION ROOT (*Taraxacum officinale*)

Natural Potassium-Sparing Diuretic

"The Weed Your Lawn Grows That Your Kidneys Love"

QUICK FACTS	
Drug class it mimics:	Diuretics (Hydrochlorothiazide, Furosemide) — WITHOUT potassium depletion
Active compounds:	Taraxasterol, sesquiterpene lactones, potassium, inulin
Recommended dose:	1-2 cups dandelion root tea daily OR 500-1000mg root extract
BP mechanism:	Promotes kidney excretion of excess sodium and water while preserving potassium
Time to effect:	Diuretic effect within hours. Sustained BP benefit over 2-4 weeks.
Critical advantage:	Unlike HCTZ, does NOT deplete potassium, magnesium, zinc, folate, B6, or CoQ10

HOW IT WORKS:

Dandelion root promotes kidney filtration and sodium excretion — the same mechanism as pharmaceutical diuretics. But here is the critical difference: drugs like hydrochlorothiazide (HCTZ) flush potassium, magnesium, zinc, folate, B6, and CoQ10 from your body alongside the sodium. Dandelion root actually CONTAINS potassium — so it promotes fluid excretion while replenishing the very mineral that pharmaceutical diuretics deplete. It also supports liver detoxification and contains inulin, a prebiotic fiber that feeds beneficial gut bacteria.

THE KEY STUDIES:

Traditional use of dandelion as a diuretic has been confirmed in modern research. A pilot study found that dandelion leaf extract significantly increased urinary frequency and volume within hours of consumption. Its high potassium content (over 4% by dry weight) makes it one of the few natural diuretics that does not create electrolyte imbalance. The European Medicines Agency has approved dandelion as a traditional herbal medicine for urinary conditions. Its use in traditional Chinese, Ayurvedic, and Native American medicine spans centuries.

HOW TO USE IT:

Tea: Steep 1-2 teaspoons of dried dandelion root in boiling water for 10-15 minutes. Drink 1-2 cups daily. Roasted dandelion root tea has a rich, coffee-like flavor — excellent caffeine replacement.

Extract/Capsules: 500-1000mg root extract daily. **Fresh:** Young dandelion greens in salads are mineral-rich and bitter — stimulating digestive function.

CAUTIONS: Diuretic effect means increased urination — stay hydrated. May interact with lithium and some antibiotics. Avoid if allergic to ragweed/chrysanthemum family. May enhance effects of diuretic medications.

#9 | POMEGRANATE (*Punica granatum*)*Natural ACE Inhibitor + ARB +
Vascular Antioxidant**"36% ACE Reduction in 2 Weeks — From a Fruit"*

QUICK FACTS	
Drug classes it mimics:	ACE Inhibitors (Captopril) + ARBs (Losartan) + Vascular Antioxidant
Active compounds:	Punicalagins, ellagitannins, anthocyanins, punicic acid
Recommended dose:	50-240ml pomegranate juice daily OR 500-1000mg extract
BP reduction:	36% ACE activity reduction documented in 2 weeks at 50ml/day
Time to effect:	ACE inhibition measurable within 2 weeks of daily consumption
Bonus effects:	Reduces arterial plaque thickness (atherosclerosis reversal documented)

HOW IT WORKS:

Pomegranate is one of the most potent natural ACE inhibitors ever studied. Its punicalagins — unique compounds found almost exclusively in pomegranate — provide 36% inhibition of ACE activity. This rivals the potency of some pharmaceutical ACE inhibitors. Additionally, pomegranate compounds interact with the AT1 receptor (the same target as ARB drugs like losartan), providing a second layer of RAAS modulation. Beyond blood pressure, pomegranate is one of the most powerful vascular antioxidants in nature — studies have shown it can actually REVERSE carotid artery plaque thickness over time.

THE KEY STUDIES:

A clinical study found that 50ml/day of pomegranate juice reduced ACE activity by 36% in just 2 weeks. A 3-year study found that pomegranate juice consumption reversed carotid artery plaque by 30% while the control group's plaque INCREASED by 9%. Multiple trials confirm pomegranate's ability to reduce oxidized LDL, improve endothelial function, and lower systemic inflammation markers. A meta-analysis confirmed significant SBP reduction with pomegranate supplementation.

HOW TO USE IT:

Juice: 50-240ml (2-8oz) of 100% pomegranate juice daily. Look for pure, unsweetened juice — not "pomegranate-flavored" blends with added sugar. **Extract:** 500-1000mg pomegranate extract capsules daily. **Fresh fruit:** Eat the arils (seeds) as a snack or in salads. The juice concentration is highest in the arils. Can be combined with beet juice for a powerful NO + ACE-inhibiting morning drink.

CAUTIONS: May interact with ACE inhibitors and ARBs (additive effect — monitor BP). May interact with statins (pomegranate inhibits some CYP450 enzymes). Juice is high in natural sugars — diabetics should use extract form. May interact with warfarin.

#10 | ASHWAGANDHA (Withania somnifera)*Adaptogen + Cortisol Reducer +
Endothelial Support**"The Cortisol Crusher — 28-30% Stress Hormone Reduction"*

QUICK FACTS	
Primary mechanism:	Adaptogen — reduces cortisol (the stress hormone that drives BP up)
Active compounds:	Withanolides (withaferin A, withanolide D), alkaloids, saponins
Recommended dose:	300-600mg KSM-66 or Sensoril extract daily with food
Cortisol reduction:	28-30% reduction in serum cortisol levels (gold-standard RCT)
Time to effect:	Anxiety/sleep: days. Full cortisol reduction: 2-4 weeks.
History:	3,000+ years of use in Ayurvedic medicine. Name means 'strength of a horse.'

HOW IT WORKS:

Ashwagandha is classified as an adaptogen — a substance that helps your body resist and adapt to physical, chemical, and biological stress. It works primarily by modulating the hypothalamic-pituitary-adrenal (HPA) axis — the master stress response system. By calming the HPA axis, ashwagandha reduces cortisol output by 28-30%. Since cortisol directly drives vasoconstriction, sodium retention, sympathetic activation, insulin spiking, and inflammatory cascading — a 28-30% cortisol reduction translates directly into lower blood pressure through EVERY one of those pathways simultaneously. Ashwagandha also improves endothelial function directly — research shows improved flow-mediated dilation (how well vessels expand in response to blood flow).

THE KEY STUDIES:

A randomized, double-blind, placebo-controlled study (gold standard) in the Indian Journal of Psychological Medicine found ashwagandha reduced serum cortisol by 28-30% compared to placebo. A study in the Journal of Dietary Supplements found ashwagandha improved flow-mediated dilation — a direct measure of endothelial function. A PLOS ONE study found significant improvement in sleep quality and reduced sleep onset latency — better sleep means better nocturnal BP dipping and vascular repair.

HOW TO USE IT:

Take 300-600mg daily with food, preferably morning or early afternoon. Look for **KSM-66** or **Sensoril** extracts — these are the most clinically studied standardized forms. Start at 300mg for the first week, then increase to 600mg if tolerated. Effects on cortisol build over 2-4 weeks — be consistent. Many people notice improved sleep and reduced anxiety within the first few days.

CAUTIONS: Avoid if pregnant or nursing. May increase thyroid hormone levels — caution with thyroid conditions (hyper- or hypothyroid). Avoid with immunosuppressant medications. May enhance effects of sedatives and anti-anxiety medications. Some people experience GI upset — take with food.

THE STACKING GUIDE — HOW TO COMBINE THESE HERBS SAFELY

Each of these 10 herbs targets a different mechanism. When combined thoughtfully, they create a compounding effect — similar to how a cardiologist prescribes multiple medications targeting different pathways. The key is to add **ONE** at a time and monitor your BP daily so you can see each herb's individual impact.

WEEK	ADD THIS	WHY THIS ORDER
Week 1	Magnesium (300-400mg before bed)	Foundational — corrects the most common deficiency first. Improves sleep immediately.
Week 2	Hibiscus Tea (2-3 cups daily)	Adds ACE inhibition + mild diuretic. Easy to integrate. Pleasant to drink.
Week 3	Garlic (2 cloves OR 600mg aged extract)	Adds 4 mechanisms at once. Stacks powerfully with hibiscus on ACE pathway.
Week 4	Ashwagandha (300-600mg morning)	Attacks cortisol — the hidden driver. Especially important if stress-related BP.
Week 5+	Add remaining herbs based on YOUR needs	Hawthorn if HR is high. Beet juice for nitric oxide. CoQ10 if on statins/beta-blockers. Pomegranate for vascular protection.

CRITICAL RULE: If you are on blood pressure medication, adding natural supplements can cause your BP to drop significantly. This is the GOAL — but it needs to happen under monitoring. Check your BP daily. If readings are consistently LOW (below 110/70) or you experience dizziness, lightheadedness, or fatigue, contact your doctor about reducing your medication dose. **Do NOT stop meds on your own.** Let the data guide the conversation.

Your body is a temple. Start treating it like one.

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This guide is part of the BP Reset Kit from BraveWorks RN.